

## SECTION III

# Assessment

Before we describe *how* an IFS therapist might assess an addictive process, we want to comment on a couple of concerns about the standard assessment approach for addictions. First, many addiction treatment programs, mental health clinics, and hospitals arrange their intake process around a traditional set of questions that assess the individual's use of drugs, alcohol, food, gambling, cybersex, and other potentially problematic preoccupations. These questionnaires have a pointed emphasis on addictive behaviors and often make no inquiry into the ways the client is functional and productive. As a result, they likely set the client up for internal backlash after the intake.

Second, clients who struggle with addictive processes are likely to go through many chapters of treatment, so by the time they get to your door, they may have been in treatment before, and they may have been told they couldn't be helped or that they didn't really want help. And if they reveal their addictive process right away, you may believe you should refer them to an addiction specialist. This is the norm. Treatment providers who are trained in traditional psychotherapy approaches typically try to avoid working with clients who engage in addictive behaviors, particularly those who use substances.

The prevailing view about substance use is that people who use substances need specialized help. As a result, even long-term clients who reveal an addictive process are often referred out (Interlandi, 2022). The therapists who refer out mean well and, on its face, their decision allows the client to get help from an expert who knows how to navigate addictive processes. If the client is in a life-threatening situation, stabilizing treatment is indeed the best course. But if the client has finally worked up the courage to confess a shameful secret and you send them off to start over with someone new, you have confirmed their deepest fear: *When I reveal myself, I'm rejected*. This amounts to telling a client that they must somehow "fix" the very problem they are seeking to address in therapy *before* they can receive your help.

This approach, in our view, impairs the effectiveness of clinical intervention from its first moments. For one thing, ambivalent clients may not follow through with referral suggestions, and even if they do follow through and manage to land a spot in an addiction treatment program (by no means a guarantee for people who don't have a lot of financial backing), the addiction specialists they see are likely to consider them uncooperative and lacking in willpower if they express a fear of sobriety or any reluctance to follow treatment recommendations. It's not unusual to hear treatment providers say that clients need to hit rock bottom before they will be serious about therapy. Likewise, it is often standard practice to give ambivalent clients additional hoops to jump through—for example, requiring that they complete intensive-outpatient treatment or attend a certain number of 12-step meetings—before they are allowed to have therapy. It is as if clients who use substances as part of their addictive process must *earn* the right to talk about their experience.

All this, in our view, indicates that generalist therapists have learned to fear—even dread—interactions with clients' firefighter parts, and they believe only specialists in addiction are equipped to handle treatment they view as a battle. The sequestering of substance use treatment and low expectations for success on the professional side, coupled with high drop-out and recurrence rates on the client side, all suggest that our prevailing treatment approaches aren't serving any of us very well. We believe that clients who aren't interested in total abstinence but are willing to talk about their

substance use should be welcomed in therapy. We trust that their firefighter parts have reason to fear getting help, and we also know that many other parts of the client desperately want help.

If seeing clients with substance use worries you, try thinking of it this way. Most clients struggle with thought and mood issues that affect their ability to be present and function well. And those who don't use substances are often heavily engaged in some type of self-regulating, compulsive activity, such as chronic gaming, watching porn, or overexercising. Whether or not a client is, as the saying goes, "clean and sober," therapy can be very effective. We agree that clinicians need to be clear about the limitations of their role and maintain kind, firm boundaries, but this is true of interactions with any client who is blended with a firefighter part. Referring clients to a specialized treatment program is only imperative when their substance use is life-threatening, but this does not include the majority of clients. In IFS, we commit to collaborating with clients, reframing addictive processes, and articulating hope.

## CREATING A CONTEXT FOR COLLABORATION AND HOPE

Hospitals, clinics, and addiction treatment programs have standard assessment forms that clients fill out in the context of a larger process called an intake evaluation. A staff member or trainee walks the client through the assessment questions to get as much factual information about them and their history as possible. The interviewer is a stranger and an evaluative authority figure who the client may never see again.

We have a bone to pick with this approach. A fact-finding interview with a narrow line of questioning (which coaxes tell-tale information about self-destructive behavior in excruciating detail) summons a fragile, shamed person to the punitive court of self-judgment. It's intrusive and it establishes a hierarchical relationship that can easily evoke fear and defensiveness. It also fails to explore the ways in which the client is productive and tells us little about how they are navigating inner conflict, which is a crucial feature of addictive processes. The standard assessment approach puts the interviewer in a one-up manager position with authority over the one-down, troubled client and is a disservice to both parties. In these ways, it is antithetical to the IFS approach.

In our first therapy session with a client, we are careful not to start by urging them to expose their most shame-inducing secrets. Just like the client and the interviewer in a standard assessment, the client and the therapist are initially strangers to each other. Therefore, in IFS therapy, our first aim is to create a safe, collaborative context. We view assessment as a process that occurs over time. Instead of launching into an investigation, we get curious, avoid assumptions, and ask a lot of open-ended questions—and we continue in this vein in every session thereafter. We learn the client's history and assess their functioning over time. We understand that larger institutions create standard criteria for evaluation in order to maintain a certain level of care and to ensure that clients get what they need, so we don't mean to minimize the challenges of changing the standard approach. But we do have strong views about what works best for clients.

The importance of a first session is that it leads to a second session. We can't help people if they don't show up. In IFS, we take a nonjudgmental stance and offer compassion from the beginning, recognizing that the client's willingness to disclose their struggles is hard-won and courageous. As therapists, we aim to embody what we call the 5 P's: Our patience, perseverance, and perspective are reassuring and calming—while our presence and playfulness are engaging. We notice the client's parts as they operate in the three categories of manager, firefighter, and exile. We offer a reframe to their struggles by eliciting and validating the positive intentions of polarized protective teams. And, above all, we offer hope: *You can feel better, and I can be your guide.*

### 5 P'S ROLE OF THE THERAPIST

- **Patience:** We know that building trust takes time.
- **Perseverance:** We offer hope and stay on message.
- **Perspective:** We look at how far a client has come, not how far they have to go.
- **Playfulness:** We use humor as appropriate to ease tension.
- **Presence:** We are fully engaged with the client.

Research shows that the quality of the therapeutic relationship is directly related to the outcome of treatment, and this is particularly important for clients who engage in addictive processes (Miller & Rollnick, 2013). When the therapeutic relationship is safe, the client can make solid inner connections. In IFS, the therapist's Self and the client's Self are two healers in collaboration, taking the bird's eye view of addiction treatment, looking at the whole system, and seeing the totality of the client's progress over time. We trust that the client's story will unfold and that they will reveal new information about their pattern of use as their protectors feel comfortable. When this happens, we take it as a very positive sign. We model the respectful, collaborative relationship style that we know warring parts can develop as exiles heal in relationship with the client's Self.

### Therapist Exercise

## Try Banning a Soothing, Distracting Firefighter Part to See How It Feels\*

The purpose of this exercise is to notice and identify which parts of you activate when you are permanently deprived of a favorite activity or behavior. This will help you understand the reluctance and fear many clients exhibit when they are told they must get sober or never engage in a particular activity ever again. Remember that all parts serve a purpose, and for clients with addictive processes, the prospect of banning a soothing or distracting firefighter can be frightening.

First, get comfortable. You can lie on the floor or sit up if you prefer. Begin by focusing on your breathing. If it feels comfortable, put one hand over your heart and the other on your belly. Take a few moments to breathe deeply into these two emotional centers. When you're ready, think of an activity you particularly enjoy. It might be something like watching funny movies, cooking, eating a favorite food, exercising, walking in the woods, or reading. Don't choose an activity or food item that is controversial for your system. Pick one that brings you genuine enjoyment without any conflict attached to it.

Now, watch yourself engaging in this activity. Notice your facial expressions and body language. When you're ready, connect to the part associated with this behavior. How much does it enjoy engaging in this behavior? How important is the behavior to the part? Perhaps it feels very important, somewhat important, or even unimportant.

Then ask the part if it is willing to play a game that may cause some distress but will teach you some important information. If it agrees, say that the game is beginning, and tell the part that it can never engage in this favorite behavior again. Not ever. If the part tries to negotiate for more time with this behavior, be very firm and insist that the behavior can never happen again. It is completely over.

Notice any reactions to this news in your body, including any thoughts as you say, "No, never again." If any parts protest to the ban, tell them they are bad if they can't agree to stop the behavior. You may notice some activation in your belly that feels like worry, anxiety, or fear. You may notice a thought response along the lines of: *Hey! You can't do that.* Perhaps you notice disbelief. Maybe a part says, "Don't worry, I'll still find a way" or "You can't tell me what to do." Connect to the parts who react and ask how this change would affect them. You may want to ask what concerns they have about giving up this activity. Again, tell them if they can't stop, they are being bad.

Finally, reassure all your parts that this was just an experiment and that it is actually fine for them to continue doing what they enjoy. Notice what that statement feels like in your body, including any thoughts or comments that come up. Take a few minutes to write about your experience. How did this experiment in deprivation feel to your system? What was it like for your parts to be told what to do and to have no recourse? How did it feel to be judged when they objected? Finally, how might this experience inform or support you in connecting with a client who comes to you for help with an addictive process?

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\* The concept for this exercise originated from IFS Senior Trainer Mary Kruger.

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### Therapist Exercise

## Explore Your Reaction to the Words *Addict* and *Addiction*

This exercise is designed to mildly activate any parts of you that hold conscious or unconscious bias about the words *addict* or *addiction*. Since these terms are commonly used in the field of substance use treatment, this exercise is designed to help you explore any unseen reactivity that these words elicit. If you think it will be too activating for you, do not do this exercise alone. Invite a friend or colleague to do it with you and support each other.

Find a quiet place and make yourself comfortable, taking a few minutes to adjust your position until you feel physically at ease or at rest. If it feels okay, close your eyes, inviting even your eyes to rest and enjoy doing less. Then focus on your breathing, and try extending your inhalations and exhalations to the count of seven, eight, or nine. Notice your deeper breath as your body settles. Take time to enjoy this feeling of self-regulation.

When you are ready, focus internally on your thoughts and emotions. Take your time simply focusing on your inner experience. Now say the word *addict* to yourself a few times or repeat the word *addiction*. As you do this out loud or internally, notice your body, feelings, and thoughts. Notice any images that come to mind. Perhaps a real or an imagined “addict” came to your mind’s eye. If so, how do you feel toward that person? What do you notice yourself thinking about that person? Do you have the urge to get away from that person or take care of them?

Maybe the word itself, without any images, activates some parts in your system. Do these parts feel sad, angry, fed up, frustrated, hopeless, or helpless? Notice all the parts who react to the words *addict* or *addiction*. Say hello to these parts, and if it feels right, be curious and ask them to share the origin of their reaction to those words. When you understand, let them know.

When your exchange feels complete, appreciate them for sharing with you. Then bring your attention back and take some time to draw or journal about your experience.

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## FROM STANDARD ASSESSMENT TO IFS ASSESSMENT

The majority of standard addiction questionnaires range from 6 to 25 questions in length and are focused solely on assessing the client's addiction symptoms. These forms ask clients to report on the frequency of their addictive behavior, increases in patterns of use, levels of preoccupation, mood changes, and any associated impact on their ability to complete daily tasks. They also ask about negative feedback from partners and family members. Some ask if there is any history of addiction, violence, abuse, or neglect in the client's family of origin. All this scrutiny at once, as we've suggested, is potentially shaming and overwhelming for new clients—not only for those who have been victimized at some point in their lives but also those who have been the perpetrator. There is a huge overlap between substance use and domestic violence, and while we don't minimize the impact of violent parts, it never pays to exile a part, even one who is guilty of a transgression (Schwartz, 2016).

In IFS, our very first intervention is an optimistic, nonjudgmental attitude. If we are patient and open-minded, compulsive firefighter parts will talk with us. We don't need to minimize the deleterious effects of their behavior or give these parts a pass in order to be interested in the benefits they pursue. And on this topic, firefighters have a lot to say. Clients need to understand how they have benefited from firefighter behaviors and how each behavior is a clue to an unmet need. In IFS, we call these clues *trailheads*, and we view them as an invitation that says, "Come this way!" As we go down that path in therapy with compassionate, mindful attention, the client hears from parts who kept them alive in tough times, and they start to build internal relationships that will heal their attachment wounds.

As we guide clients to befriend their parts and understand their motives, we find out who is ready to try something new—and who isn't. We expect ambivalence. If the client is in our office after years of trying to change, we know it hasn't worked out yet. But their willingness to try again indicates that some (or many!) of their parts are ready to try again. At the same time, we know they have other parts who feel differently. Some are afraid of feeling too much, some are afraid of people, and some enjoy using. Mixed feelings and multiple perspectives are normal, and together they call for inquiry. People who are caught in a cycle of physical cravings, emotional pain, and all-too-temporary relief desperately need to inquire inside, as this sustained, compassionate reflection can lead to healing.

Physician and author Gabor Maté suggests setting aside the question "Why this addiction?" to instead ask, "Why this pain?" Similarly, IFS therapists set aside the question "Why are you invested in self-destructive behavior?" to ask, "How are your parts trying to help you?" We abstain from the question "Why are you so checked out?" to ask, "What are you afraid would happen if you didn't use this substance (or engage in this activity) anymore?" We want to hear about the needs that lie underneath extreme behavior, and we get that information by including all of the client's parts. IFS offers a set of clear, kind interventions that avoid taking sides and start where the client is today.

### Advice for Therapists Who Create Their Own Assessment

If you have the freedom to formulate your own initial evaluation, we recommend not using a lengthy initial assessment of the client's history of addictive processes and their attempts to be abstinent. Your goal for the first contact is to engage the client. Remember, you want people who seek help to be willing to come back and get help.

While assessing someone's addictive process, it's also important to recall that different clients will initially have different end goals. Clients who reveal their firefighter parts but don't talk about their managers will certainly be allergic to the notion of abstinence. You can locate this aversion by asking what they've been told about abstinence. Many will have heard that any other outcome is failure. This

question will also let you clarify your approach. The IFS therapist aims to help clients be in relationship with their parts—not to control them.

If a client needs prompt intervention to stay alive, there isn't much room for negotiation. But even (or especially) then, it helps to assert that harsh managers and extreme firefighters all mean well, that exiles can be healed, and that the client is welcome to continue therapy after their inpatient stay. This kind of warm reassurance can go a long way toward winning cooperation.

### **Advice for Therapists Who Are Required to Take a Traditional Addiction History**

If you work in an institution that uses an addiction history questionnaire, you can sidestep some of the problems mentioned earlier by joining with the client and giving yourself a bit of distance from the institution. For example, you might say something like “The clinic needs us to fill out this form. After that, we can talk about what you need and what you’re looking for.” After filling out the questionnaire, you can survey the client’s parts, both the ones who help them function (often quite well) in various arenas and the ones who engage in addictive behaviors. As you go about this evaluation, you’ll want to assess—just for yourself at this point—the role of the part who is taking the lead in this first interview. Is it a firefighter, a manager, or an exile?

#### **HOW THE IFS THERAPIST STARTS THERAPY**

- Welcome the client’s full system of parts and talk with any blended parts by using implicit direct access.
- Ask why they seek help now, and listen for conflicting parts in their answer.
- Support their courage in reaching out.
- Dive into the language and perspective of the model by reflecting back with parts language (e.g., some parts want to control the behavior of other parts, some parts worry about their functioning, some parts like to use).
- Explain that you see the client’s parts as an inner community (or family) that’s engaged in a repetitive battle over how to stay safe.
- When a client presents with urgency and seems unambiguous about addressing their addictive behaviors, assume you are only hearing one side in an ongoing argument.
- Although it may feel counterintuitive, engage with parts who want to keep using. They are nervous and suspicious about seeking help. They need to know that you care about their perspective.
- Invite skeptical parts on both sides of this polarity: the ones who are reluctant to trust and the ones who are tired of the chaos, instability, and health concerns.
- Assume that parts are burdened by doubts and may be terrified about trying once again to get help.



## WHEN A FIREFIGHTER PRESENTS FIRST

Firefighters are good at avoiding direct answers and may leave you with the distinct impression that the client is minimizing their level of use or other addictive behavior. A firefighter might, for example, rationalize the client's situation as a single, current crisis that will never happen again. It might also insist that the overblown concerns of other people are the real problem, outright deny having a problem, or complain about whoever mandated the client to get help. Whatever their strategy, this no-big-deal attitude puts the therapist in a bind. Do you take a firmer stance and insist on answers that sound more honest? Or accept the futility of a power struggle and go with vague half-answers of doubtful truthfulness?

If you assume a more confrontational stance with firefighters, you automatically form an alliance against them—and *with* the client's manager parts. But if you don't challenge them at all, you're excluding managers from the conversation. Neither option is good. Just as excluded managers will eventually step in with a vengeance, firefighters will take over at some point if they are dismissed. So what do you do? Luckily, you don't have to join with managers or outfox firefighters. It's easier and more effective to simply ask the client if they ever hear from the other side.

For example, Roger, a thirty-eight-year-old, Irish American, heterosexual, married, cisgender man, was employed in construction management. He was depressed, and his use of alcohol and cannabis had recently increased dramatically. His wife insisted that he go to therapy. This was his third session.

### WORKING WITH A FIREFIGHTER



**Therapist:** How are you, Roger?

**Roger's Firefighter:** Not great really. My wife is not talking to me. We got into it last night because I was hanging out in the garage and she is so sick of it.

**Therapist:** Sounds stressful. When you say, "hanging in the garage," do you mean you were drinking?

**Roger's Firefighter:** Yeah, I was out there till midnight or something.

**Therapist:** Uh-huh. So, you headed out there after work or...?

**Roger's Firefighter:** Well, I went into the house when I got home. But then, yeah, I had a case in my truck and, ya know, some weed, and I went out to the garage. I've got a TV out there and everything.

**Therapist:** Right. So by the time you went back into the house, everyone was asleep and you were pretty loaded.

**Roger's Firefighter:** Yeah. I slept on the couch and when I got up for work, my wife just started ragging on me. She thinks I never see the kids and she has to do everything. She says I'm embarrassing her.

**Therapist:** When you're doing your thing and not with her and the kids, she is angry and maybe disappointed, too.

**Roger's Firefighter:** I really don't need to hear that shit. I had a huge thing going on at work and I don't need to get screamed at before I'm half-awake in the morning.

**Therapist:** I hear you. It's stressful to be yelled at. And you were probably not feeling that great.

**Roger's Firefighter:** No. I wasn't! She's not considerate. She could lay off for once!

**Therapist:** Right. I do get that. It's like she's angry with you so often. Can I ask one question? (*Roger nods.*) When your wife is accusing you of not being around, of getting loaded and not being with the kids, have you ever said the same thing to yourself? Maybe when you first wake up in the morning?

This is an unblending question. The therapist has been speaking directly with the firefighter—without directly naming the part and by instead using the pronoun *you*, which means this is *implicit direct access*. The therapist now asks the client to notice other parts.

**Roger's Firefighter:** Say what to myself? I don't have time to say shit before she starts in on me.

**Therapist:** Right, this morning you woke up to an angry wife. But my question is more general. If not this morning, have you ever had a similar thought? Like you were shaving and you looked in the mirror and thought, *Okay, maybe I am hitting it too hard*. Or you had a kind of a guilty feeling about the kids, like that you haven't been around for them much. Is that possible? Have you ever had your own worries about all this? The therapist persists.

The therapist persists.

**Roger's Firefighter:** Well with her going on and on like I'm doing this every day, which I do not—

**Therapist:** (*Therapist interrupts.*) Yes, I get it. You feel like your wife can't see any of the good things about you anymore. But aside from her, aside from her anger with you, I'm wondering if *you* have ever said something similar to yourself, like *I need to cut back* or *That headache is just too big when I get up in the morning*.

The therapist interrupts the firefighter, parenthetically acknowledges its point, and persists.

**Roger:** Ever? You mean have I ever felt shitty about drinking and getting high and all that?

Roger's Self starts to come online as he gets a little space from the firefighter, enough to acknowledge that he has manager parts.

**Therapist:** Right, have you ever worried a little bit about yourself?

**Roger:** Well, yeah. I mean it's not the greatest to spend the night in the fucking garage getting fucked up. I mean, yeah, it's not like I want to be doing this for the rest of my life. But she acts like that's all I do, like I live out there. And last weekend I went to see her sister with her and the kids—

The firefighter jumps in again.

**Therapist:** (*Therapist interrupts.*) I know. I do know that your wife has her own perspective and it doesn't match yours. But am I hearing it correctly that, aside from her view, you have your own feeling, your own sense of things, which, at times, says you are using a little more than is healthy for you?

The therapist again interrupts the firefighter, parenthetically acknowledges its point, and persists.

**Roger:** Sure, there have been days I feel bad about it.

Roger acknowledges his polarity.

**Therapist:** Okay. So what I hear is that you have a part, or maybe a few parts, who like to get high and just want to be left alone to do their thing, right? (*Roger nods.*) And you have a whole other set of parts who can be critical of that, who say that those drinking parts go too far, that using is causing problems.

The therapist lays out the polarity between Roger's parts.

**Roger:** Okay. I guess that's true.

**Therapist:** It's not an easy way to live, is it? Going back and forth between using and then getting mad at yourself and trying not to use. And then feeling like you have to use again, and so on.

The therapist paints the big picture.

**Roger:** Yeah, that's my life.

Roger is more unblended now and better able to take the bird's eye view.

**Therapist:** We can do something about that. If you're up for it, we can try something right now. (*Roger nods.*) Okay, let's set it up. Invite all these parts to come sit with you at a big table. You sit at the head of the table and put the substance-using parts who want time to chill and like getting loaded on one side. Then put the other parts—the ones who aren't happy about getting high—on the other side.

When protectors are polarized, a conference with both sides helps everyone to unblend. It's like a poker game where everyone keeps their hands on the table.

**Roger:** Okay. Now what?

**Therapist:** If you had a work meeting, and the people on one side of the table wanted to move forward with a project but the people on the other side didn't, you'd listen to both sides, right? (*Roger nods.*) So let each side know that you're going to listen to them all. They all have important views. Ask if they're willing to take a break from fighting with each other and shift over to talking with you, just for now.

**Roger:** Okay. But it's not like they trust me.

**Therapist:** Of course not. Tell them you see their distrust, and, going forward, you're going to earn their trust because they're all important to you.

**Roger:** Well... (*He chuckles.*) here's one thing they can *all* agree on: We've got a way to go here!

**Therapist:** Very true. We do. But you're on your way now. This is new. Are you willing to stay with them so they feel less alone?

**Roger:** Okay, okay. Yeah, they do feel alone. They like me being here.



If the client has been mandated to treatment, which happens often to people who engage in costly compulsive behaviors, questions like “What brings you to therapy?” may elicit complaints and defensiveness from a blended firefighter part, as we saw with Roger, or even paranoia about the therapist’s motives rather than hope and a willingness to engage. Roger began this session by focusing on his wife’s anger rather than his behavior and its impact on her and their kids. His angry, defensive, substance-using parts spoke as if her anger was unjustified. They expressed no empathy and had no interest in understanding how his absences and drinking affected the family. Nor were they inclined to see that using was a problem for him.

The therapist could have justifiably supported his wife’s point of view, but that would mean polarizing with firefighters who weren’t yet feeling comfortable enough to be honest or ask for help. Alternatively, the therapist could have challenged Roger when he said his wife’s wrath was out of line, hoping he would see reason and admit that his drinking was causing problems. But with both of those options, the therapist would have been challenging Roger’s firefighter stance.

This IFS therapist had a different aim, which was to help Roger discover his other team: the managers who felt responsible for and concerned about his substance use and who were worried about his family. In his initial firefighter narrative, Roger complained about other people, and his managers weren’t audible. But the therapist knew they existed and were trying to control his drinking. The therapist knew that these two protector teams were fighting with each other inside. To help Roger stop projecting this internal conflict into his relationships with his wife and kids (or his relationship with the therapist), the therapist stayed curious and asked if Roger was sometimes angry with himself. When he finally admitted to some self-criticism, the therapist was able to help him meet his polarized teams. As Roger sat at the head of the table as a third party who could welcome the positive intentions of both sides, he was able to collaborate with the therapist and begin to build Self-to-part relationships with his managers and firefighters.

## WHEN A MANAGER PRESENTS FIRST

The previous interaction with Roger illustrates how we dig around for evidence of concern when we talk with blended firefighter parts, but if we’re talking with a manager, we take another tack, which is to validate their concerns and check for exhaustion and a willingness to try something new. Consider the case of Helen, a forty-eight-year-old, Mexican American, single, cisgender woman, who had been bulimic and in therapy for most of her teenage years. She was no longer bingeing and purging, but she was intensely worried about smoking the occasional cigarette and what she called “unhealthy eating” at night. This eating was interfering with her sleep and her ability to function during the day. She also reported, by way of background information, that she was spending most weekends with old friends drinking, taking pills, and using cannabis.

Many clients engage in more than one firefighter activity. Disordered eating and substance use are a common combination. Since it sounded like Helen’s eating issue was relatively mild, a part of the therapist wanted to prioritize Helen’s chronic, heavy weekend substance use. However, the therapist set that sense of imperative aside (without ignoring it) in order to meet Helen in the here and now, help her understand her system, and get some clarity and immediate relief.

## WORKING WITH A MANAGER



**Therapist:** So, Helen, I'm hearing that your work feels pretty stressful, and on weekends you really let everything go. Is that about right? (*Helen nods.*) I appreciate your sharing that with me. Maybe later on at some point, we can talk more about how that is working for you. I hear, though, that the eating at night is most important to you now. Can you tell me more about it?

**Helen's Manager:** Yeah, the weekends are *fine*—I need my weekends!

**Therapist:** I hear you. You need your weekends.

Helen is blended with her substance-using firefighter parts. The therapist doesn't challenge them but takes note internally that Helen needs to use.

**Helen's Manager:** But I have to get over this eating thing! I was so tired at work today because last night I got up at 2 a.m. to eat cereal, which is ridiculous. So then I smoked a few cigarettes, which I have to stop because it's disgusting. And it took me till 4 a.m. to get back to sleep. This has been going on for months. I can't take it anymore! And it's going to happen again tonight.

**Therapist:** I hear you're worried and stressed. This feels odd and it affects you at work the next day. But you can't get a handle on it.

The therapist is speaking directly to Helen's worried, frustrated managers, which reflects direct access. Helping them feel understood will help them unblend.

**Helen's Manager:** It's screwing me up, that's for sure! I'm sick of it. It's so disgusting. I try to make myself go back to sleep, but I can't. Nothing works. I need to get over this. It shouldn't be this hard!

**Therapist:** Let me reflect back what I'm hearing and the parts I'm noticing. (*Helen nods.*) I'm hearing a team of parts—we'll call them your manager team—who are very concerned about the nightly eating. One critical part closely watches all that eating and hates it. A judging part is also disgusted by the eating and the smoking. Then I notice a very urgent, maybe even desperate controlling part that says this has to stop. And I think I hear even another harsh and perfectionistic part who says it should not be so hard to stop. These parts are driven to change the parts who are eating and smoking in the night; we'll call them your soother team.

**Helen's Manager:** Yeah, they need to change all right.

Helen is blended with the manager team.

**Therapist:** But the eating and smoking parts don't seem to be listening, is that right? The soother team is back at it every night, no matter how much the critical, judging, and perfectionistic parts yell at them. And maybe there's one more little part in the background who gets scared and feels hopeless about getting it right? (*Helen nods and shrugs, as this is all very familiar.*) Let's invite them all to come sit

with you at a big conference table. You sit at the head, and they can sit opposite each other. Do you see them? And do they see you? (*Helen nods.*) If the scared, hopeless one wants to be there, put it across from you at the foot of the table for now. Which team needs your attention first, Helen?

**Helen's Manager:** I need to work on the eating! So, the cereal-eating part.

**Therapist:** Okay great. Let's connect with that part. Can you see yourself eating cereal at night?

**Helen's Manager:** I see myself sitting on the couch with a big bowl of cereal.

**Therapist:** Okay, so just sit back and observe her. What is her expression? What is her body language? How do you feel toward her, as you watch?

**Helen's Manager:** She's pathetic. She just sits there shoveling it in. It's depressing to look at her.

The therapist notices their own parts cringing in response to this harshness.

**Therapist:** Hmm, okay, so you have a part who feels judgmental toward the cereal-loving one. It's disgusted and critical.

**Helen's Manager:** Yes, because the eating is disgusting.

**Therapist:** I get it. The cereal-eater is, like, in a trance? (*She nods.*) And you can hear how hard the critic comes down on her?

**Helen's Manager:** Well, eating in the night is a terrible habit! And then because I can't stop and I'm afraid I'll gain weight, I smoke. But I hate smoking! And every night when I go to bed, I say *Maybe I won't get up and do it tonight.* But I do and I don't know why. Oh, I hate myself!

**Therapist:** Your managers are afraid for you because you're not in control in the middle of the night. And the smoking part tries to help with the risk of weight gain. Let's pause for a minute and appreciate them. Do they notice you?

**Helen:** Okay yeah, I guess they know I'm trying to listen. But they hate me.

**Therapist:** The critic does really hate some of your other parts, but it doesn't hate *you* because it doesn't know you yet. Let it know that you're listening, and you know it hates the parts who eat and smoke in the night. But you are not them. You are the one who listens to everyone. The critic is talking to you now.

**Helen:** It did think I was the cereal-eating part. Hmm.

**Therapist:** The cereal-eating part certainly does exist. Ask the critic why it needs to hate the cereal-eating part. How is it trying to help you?

**Helen:** It hates her because she is so totally into her food and it thinks she'll get worse. Like she'll start throwing up again and eating more.

**Therapist:** That makes sense. And that would be really bad. Has the critic seen the cereal-eating part be much worse than it is now?

The therapist is helping Helen tune in to the critic's intention, which is to prevent a recurrence of firefighter behavior and appreciate that the critic has seen Helen suffer from bulimia during her teenage years, so its fear is legitimate.

**Helen:** Well yeah, it's seen me worse, of course. I mean, I used to throw up all time. My eating was crazy. My life was totally messed up.

**Therapist:** That was a super tough time. Let the critic know you get it. It was a long time ago, but the critic has a point! The bingeing and purging parts were out of control and ran your life back then. You understand that, right, Helen?

**Helen:** Yeah, I know how bad that was. I get why it's trying to stop the cereal-eating part now.

**Therapist:** There is a different option now, though. If you connected with the cereal-eater and the smoker and helped them, would the critic, judge, and perfectionist be willing to let go a little bit?

**Helen:** They're not so sure about that!

**Therapist:** There's no rush. But they should know that the eating and smoking parts mean well. There's more to them than meets the eye. If we could connect with them and help them use their energy in a different way, would that be good?

**Helen:** Well, if we could, it would be good. But the critic isn't convinced. It doesn't think I have any influence.

**Therapist:** Well, you would have to demonstrate that, wouldn't you? Would they step back and let you connect with the eating and smoking parts, just for a few minutes, here in our session, to show what you can do?

**Helen:** Okay, okay. They'll let me try. They're so tired of all this.

**Therapist:** They're tired. But with you here, they're not alone anymore. (*Helen relaxes visibly.*)



In IFS, we aim for therapy to be an accepting, safe container for shaming managers, as well as shameless firefighters and exiles who feel shameful. Manager fears are completely reasonable based on what they know from the past. We assure them that the client's Self can help the firefighter team (which also has good intentions) to behave more responsibly. Helen came to this session feeling anxious about her lack of sleep and totally blended with critics who were attacking her cereal-eating and smoking parts. Although she spoke of a team of weekend warriors who were using heavily, her managers were most afraid of the unwelcome night visitors and their effect on her work performance, and we want to respect the client's priorities. As Helen experienced relief from her initial concerns, she was able to focus more directly on painful early childhood issues, eventually addressing her weekend activities as well.

## WHEN AN EXILE PRESENTS FIRST

Finally, there are times when a client presents in therapy while blended with an exiled part. You can tell that the client is blended with an exile (or is in imminent threat of being blended with an exile) if they present in a state of collapse or near collapse. They may appear crumpled, cringing, tearful, overwhelmed, or dissociated (a firefighter responding to the blended exile). If the therapist's protectors are inclined to over-function or distance from the client in these situations, the therapist will need to help their own protective parts step back first. Only then can the therapist help the client's protectors unblend so the client's Self can become available, welcoming the exile with plenty of compassion for its panicked state. To facilitate this Self-to-part connection, the therapist reflects back



what the client is communicating. The therapist may also want to pause the client and initiate a moment of unblending by inviting the client to take a breath and appreciate how much their parts have been holding all week. This step recruits the observational perspective of the Self, if briefly.

For example, Luna, a twenty-seven-year-old Italian American, cisgender, single, heterosexual woman, first came to therapy for generalized anxiety, depression, and posttraumatic stress disorder. She arrived at her twentieth session very anxious and at one point folded over in her chair, barely able to talk as she sobbed.

## WORKING WITH A EXILE



**Therapist:** I'm going to talk with this part directly, Luna. I want to speak with the part who is upset. Are you there?

**Luna's Exile:** *(After a long pause, she speaks in a small voice.)* Yes.

**Therapist:** You're welcome here. I want you to know that. Do you protect any other part? *(Luna shakes her head no.)* Would you like help? *(She nods.)* Good. Because we want to help you. I'm going to explain what you can do to make sure that happens. Okay? *(Luna nods.)* While there are parts who keep you hidden, Luna also has a Self—who is not a part—who can help you. Luna's Self can talk with the ones who keep you hidden. With their permission, she can help you. Meanwhile, we can put you in a safe place. Would that be good?

We assert the crucial difference between the client's Self and their parts while leaving the door open for the client to understand the Self in their own way.

**Luna's Exile:** *(She continues in the small voice.)* Yes.

Seeing the benefit of cooperating, the exile agrees to unblend.

**Therapist:** Great. Where would you like to be?

**Luna's Exile:** The waterfall.

**Therapist:** Good. Be at the waterfall. Do you need anything there?

**Luna's Exile:** My dog.

**Therapist:** For sure. Bring your dog. Now I'm going to talk with Luna. Are you there, Luna?

**Luna:** Yes.

**Therapist:** How are other parts responding?

**Luna:** They like it.

**Therapist:** Any concerns?

**Luna:** What if the upset part gets overwhelmed again?



**Therapist:** We'll help her stay at the waterfall until you have permission to help her.

**Luna:** Okay.



As this vignette illustrates, when an exile blends too soon, we aim to reassure it, explain the plan, and win its cooperation. This is generally not hard to do if the exile understands that cooperating will lead to help. After that, it's important to check back with protectors to be sure they're on board.